This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of yellow fever vaccine (Stamaril)

at designated Yellow Fever Vaccination Centres

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Guideline Summary: yellow fever vaccination

Designation is a precondition

- Yellow fever vaccine may only be given at a centre designated by NaTHNaC, which manages designation, training, registration, standards and audit for Yellow Fever Vaccination Centres in England, Wales and Northern Ireland. Scotland and the Crown Dependencies are managed separately.
- The vaccine may only be administered by a registered doctor, nurse, pharmacist or dentist working at the designated centre, with unrestricted registration and no pending investigation, warning, caution or suspension.
- This PGD must not be used at a pharmacy that does not hold current designation.

The vaccine

- Stamaril is a LIVE attenuated 17D vaccine. That is the single most important fact in this document: in an immunosuppressed patient the vaccine strain can replicate and cause extensive, severe and sometimes fatal infection.
- It is propagated in chick embryos, so egg allergy is relevant and anaphylaxis to egg is a contraindication here.
- A single 0.5 ml dose is given subcutaneously after reconstitution with the supplied solvent, and used immediately.
- Age: the vaccine is contraindicated under 6 months, is considered at 6 to 8 months only on specialist advice where the risk is high and travel unavoidable, and is given routinely from 9 months. The lower age limit under this PGD is 9 months.

The certificate

- The International Certificate of Vaccination or Prophylaxis (ICVP) becomes valid 10 days after a first dose. A traveller vaccinated less than 10 days before departure may be refused entry or quarantined where a certificate is required, and must be told so; the vaccine itself protects from about day 10 regardless.
- Since 11 July 2016 the certificate is valid for the life of the person vaccinated. It does not expire after 10 years.
- Where vaccination is contraindicated, travel is unavoidable and the destination requires a certificate, a Medical Letter of Exemption can be offered instead of vaccination.

Specialist advice

- NaTHNaC operates an advice line for health professionals on 020 7383 7474. Use it for any patient falling under the precautions below, and record the advice received.

Patient Group Direction

for the administration of

Yellow fever vaccine, live attenuated 17D (Stamaril)

for protection against yellow fever, at designated centres only

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacists only. Pharmacy technicians may not administer yellow fever vaccine under the NaTHNaC conditions of designation.
Training and assessment	Pharmacists must have completed training relevant to this condition, and the NaTHNaC yellow fever training required for the designated centre, to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed immunisation training in line with the National Minimum Standards and Core Curriculum for Immunisation Training ● All users must be competent in the recognition and management of anaphylaxis, and in vaccine handling and cold chain management ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005 ● All users must be familiar with Gillick competence and the assessment of consent in children and young people, and with

	the involvement of a person with parental responsibility

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation against yellow fever with Stamaril, live attenuated 17D vaccine, for individuals travelling to or residing in a country or area where yellow fever is a risk, or where an International Certificate of Vaccination or Prophylaxis is required for entry, in accordance with Chapter 35 of Immunisation Against Infectious Disease (the Green Book) and current NaTHNaC guidance. This PGD may only be used at a designated Yellow Fever Vaccination Centre.
Inclusion criteria	● Aged 9 months and over. ● Travelling to or residing in an area where yellow fever vaccination is recommended by NaTHNaC, or where a certificate is required as a condition of entry, confirmed against TravelHealthPro at the time of the consultation. ● Travelling at any interval before departure. Where a certificate is required and will not be valid in time (it becomes valid 10 days after vaccination), explain that the destination may refuse entry or quarantine, reinforce bite avoidance, and vaccinate if the traveller wishes. Record the advice. ● Valid informed consent obtained, from a person with parental responsibility where the individual is under 16 and not Gillick competent. ● Administered at a designated Yellow Fever Vaccination Centre by a practitioner authorised under that designation. ● No contraindication as set out below.
Exclusion criteria	● Aged under 6 months. Absolute contraindication because of the increased risk of vaccine-associated encephalitis. ● Aged 6 to 8 months. Outside this PGD: vaccination is only considered where the risk of transmission is high, such as during an outbreak, and travel is unavoidable. Seek specialist advice and refer. ● Confirmed anaphylactic reaction to a previous dose of yellow fever vaccine. ● Any immediate-type (IgE-mediated) allergy to egg or chicken protein, or to any component of the vaccine, that has not been shown to be outgrown. Refer to an allergy specialist or hospital-based clinic; do not vaccinate under this PGD. ● History of a thymus disorder, including myasthenia gravis and thymoma, or thymectomy for any reason including during cardiac surgery. ● A first-degree relative, meaning mother, father, full sibling or child, who has had yellow fever vaccine associated viscerotropic or neurologic disease (YEL-AVD or YEL-AND) not explained by a known medical risk factor. ● Primary or acquired immunodeficiency, or immunosuppressive therapy. This includes leukaemia and lymphoma, severe immunosuppression due to HIV, cellular immune deficiencies, chronic lymphoproliferative disorders, stem cell transplant within 24 months, chemotherapy or radiotherapy within 6 months, solid organ transplant immunosuppression within 6 months, biological therapy within 12 months, and high dose corticosteroids or oral immune modulating drugs within 3

	months at the thresholds set out in the NaTHNaC table. ● Acute severe febrile illness. Postpone until recovered, allowing 10 days before travel for the certificate to become valid. ● Pregnancy. Do not vaccinate under this PGD. Advise against travel to a yellow fever risk area; where travel is unavoidable, refer to a specialist yellow fever centre for an individual risk assessment, and offer a Medical Letter of Exemption where a certificate is required for entry. Vaccinate after pregnancy if the risk continues. ● Aged 60 years or over and travelling only to areas where WHO designates yellow fever vaccination as generally not recommended or not recommended (Green Book chapter 35 contraindication). ● Any patient falling under the precautions below where specialist advice cannot be obtained before vaccination.
Cautions including any relevant action to be taken	● Facilities and trained staff for the management of anaphylaxis must be available, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone. ● Aged 60 years and over: the risk of YEL-AND and YEL-AVD rises with age, and almost all cases occur with a first dose. Vaccinate only where there is significant and unavoidable risk after a detailed risk assessment. Travel only to areas where vaccination is not recommended is an exclusion (above). ● Women of childbearing potential: advise avoiding pregnancy for one month after vaccination (Stamaril SmPC 4.6). Record that this advice was given. ● Breastfeeding an infant under 9 months: there is evidence of transmission of live vaccine virus in breast milk to infants under 2 months. Seek specialist advice. ● Living with HIV: there is limited evidence that the vaccine may be given safely where the CD4 count is above 200 with a suppressed viral load, and the antibody response may be reduced. Seek specialist advice. ● Low dose corticosteroid or non-biological oral immune modulating therapy: long term low dose therapy is not usually considered sufficiently immunosuppressive and these patients can generally receive live vaccines. Data are limited, so specialist advice may be sought. ● Record any specialist advice received, including who gave it and when, in the consultation record.
Actions if patient is excluded or declines treatment	Explain why the vaccine cannot be given and what the alternatives are. Where a certificate is required for entry and vaccination is contraindicated, offer a Medical Letter of Exemption and explain that acceptance is at the discretion of the destination country. Reinforce mosquito bite avoidance, which is the only other protection against yellow fever. Advise on alternative provision, including their GP practice or a specialist travel clinic, and document the advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Stamaril, yellow fever vaccine (live attenuated 17D-204 strain), powder and solvent for suspension for injection in a pre-filled syringe. One 0.5 ml dose after reconstitution.
Legal category	POM

Storage	Store at +2°C to +8°C. Do not freeze. Store in the original packaging to protect from light. Reconstitute with the supplied solvent and use immediately; discard any unused reconstituted vaccine. Vaccine exposed to conditions outside the stated range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance.
Route/method of administration	• Reconstitute with the supplied solvent, mix gently, and administer immediately. • Give 0.5 ml by subcutaneous injection, usually into the upper arm. • Inspect visually before administration and do not use if the appearance differs from that described in the SPC. • Where given with other vaccines, use separate sites, preferably different limbs, or at least 2.5 cm apart, and record the site of each.
Dose and frequency of administration	A single 0.5 ml dose. The certificate becomes valid 10 days after a first dose and then lasts for the life of the person vaccinated, so routine revaccination is not required. A reinforcing dose may be given under this PGD, where there is no current contraindication, to a person whose first dose was given when aged under 2 years, during pregnancy, while infected with HIV, while immunosuppressed, or before a bone marrow transplant (Green Book chapter 35). A booster may be considered after 10 years for prolonged high-risk exposure. Record which applied.
Quantity to be administered and/or supplied	One 0.5 ml dose.
Maximum or minimum treatment period	Single dose. Vaccinate at least 10 days before travel where a certificate is required.
Disposal	Dispose of used syringes, needles and any reconstituted vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Drug interactions	Do not give on the same day as MMR; separate yellow fever and MMR by 28 days unless protection is needed rapidly, in which case give at any interval and consider an additional MMR dose. Other live and inactivated vaccines may be given at any interval, at separate sites. The immune response may be reduced in individuals on immune modulating therapy.
Adverse effects	Very common and common: injection site pain, redness and swelling, headache, malaise, myalgia and fever. Rare: hypersensitivity reactions including anaphylaxis. Very rare but serious: yellow fever vaccine associated neurologic disease (YEL-AND) and viscerotropic disease (YEL-AVD), which occur almost exclusively after a first dose and more often in those aged 60 and over. Refer to the current SPC for the full list.
Yellow card reporting	Report suspected adverse reactions to the MHRA via the Yellow Card scheme at https://yellowcard.mhra.gov.uk. Any suspected YEL-AND or YEL-AVD must be reported and the patient referred urgently. Document any adverse reaction and inform the GP.
Records to be kept	Record the following information: • that valid informed consent was given by the individual, or by a person with parental responsibility, or that a decision was made in the individual's best interests in accordance with the Mental Capacity Act 2005 • name of individual, address, date of birth and GP with whom the individual is registered • name of immuniser • name and brand of vaccine • date of administration

	• dose, form and route of administration • quantity administered • batch number and expiry date • anatomical site of vaccination • advice given, including advice given if the individual is excluded or declines immunisation • details of any adverse drug reactions and actions taken • that the vaccine was administered via PGD • the certificate number issued and the date from which it is valid • any specialist advice obtained, including who gave it and when Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed. • Adults aged 18 years and over: keep records for 8 years • Children aged under 18 years: keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Offer the marketing authorisation holder's patient information leaflet, and issue the International Certificate of Vaccination or Prophylaxis completed and signed in accordance with NaTHNaC requirements. Explain that the certificate becomes valid 10 days after this dose and then lasts for life, and that a replacement can be obtained if lost.
Follow-up advice to be given to patient or carer	Advise on possible adverse effects and their management, and to seek urgent medical attention for fever, jaundice or severe illness in the days and weeks after vaccination, mentioning that they have recently received yellow fever vaccine. Reinforce mosquito bite avoidance, which also protects against dengue, Zika, chikungunya and malaria, none of which this vaccine covers. Advise that malaria prophylaxis may still be required for the destination.

Key references

- Immunisation Against Infectious Disease (the Green Book), Chapter 35 (Yellow fever)
- NaTHNaC: Contraindications and precautions for administering yellow fever vaccine, updated 27 May 2026
- NaTHNaC Yellow Fever Zone: conditions of designation and code of practice for Yellow Fever Vaccination Centres
- Summary of Product Characteristics for Stamaril, Sanofi, current version
- MHRA Drug Safety Update: yellow fever vaccine (Stamaril) and fatal adverse reactions, extreme caution in people who may be immunosuppressed and those 60 years and older
- NaTHNaC / TravelHealthPro country-specific recommendations
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which	

this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
30/9/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development and issue of new PGD	30th July 2026

VACCINE SAFETY REQUIREMENTS, ADDED AT VERSION V002, 9 September 2026

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

These requirements were added at version 002 and have been carried forward through the clinical reviews of versions 003 and 004.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

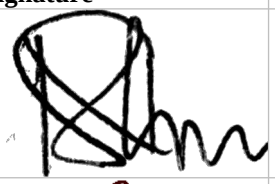
Version and change record
Version: v007
Issued: 30 September 2026
Supersedes: Version 006, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v006
Issued: 14 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The guidance summary now states the age floor: contraindicated under 6 months, specialist advice only at 6 to 8 months, routine from 9 months, which is the lower age limit of this PGD.
The duplicate 'batch number, expiry date and anatomical site' line in the records row was removed; both items remain in the list.
The introduction to the vaccine safety block no longer says that a clinical review remains outstanding; versions 003 and 004 were clinical reviews.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v004
Issued: 11 September 2026
Supersedes: Version 003, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Pregnancy moved from the cautions to the exclusion criteria: not vaccinated under this PGD, referred to a specialist centre, Medical Letter of Exemption offered. A caution that says vaccination 'can be considered' left a pharmacist to make a live-vaccine-in-pregnancy decision that is a specialist's.
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change

records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v002
Issued: 9 September 2026
Supersedes: the previous signed version of this document.
Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Observation after vaccination.
Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.
Version: v003
Issued: 10 September 2026
Supersedes: Version 002, 9 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated.
Pharmacy technicians removed from the professionals covered: the NaTHNaC conditions of designation restrict administration to registered doctors, nurses and pharmacists, as the document's own guidance summary states.
Egg allergy: the Stamaril SmPC contraindicates any hypersensitivity to egg or chicken protein, not only anaphylaxis. The exclusion now says so and directs to a specialist clinic.
Aged 60 or over travelling only to areas where vaccination is not recommended: moved from a caution to an exclusion, as the Green Book lists it as a contraindication.
Last-minute travellers: the 10-day rule was written as an inclusion criterion, which barred them. The Green Book says vaccinate and counsel. Reworded.
MMR: the Green Book requires yellow fever and MMR to be given 28 days apart and never on the same day; the previous text said the opposite. Other vaccines may be given at any interval.
Reinforcing doses: all five Green Book groups now listed (first dose under 2 years, in pregnancy, with HIV, when immunosuppressed, before bone marrow transplant).
Women of childbearing potential: avoid pregnancy for one month after vaccination (SmPC 4.6). Added.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.